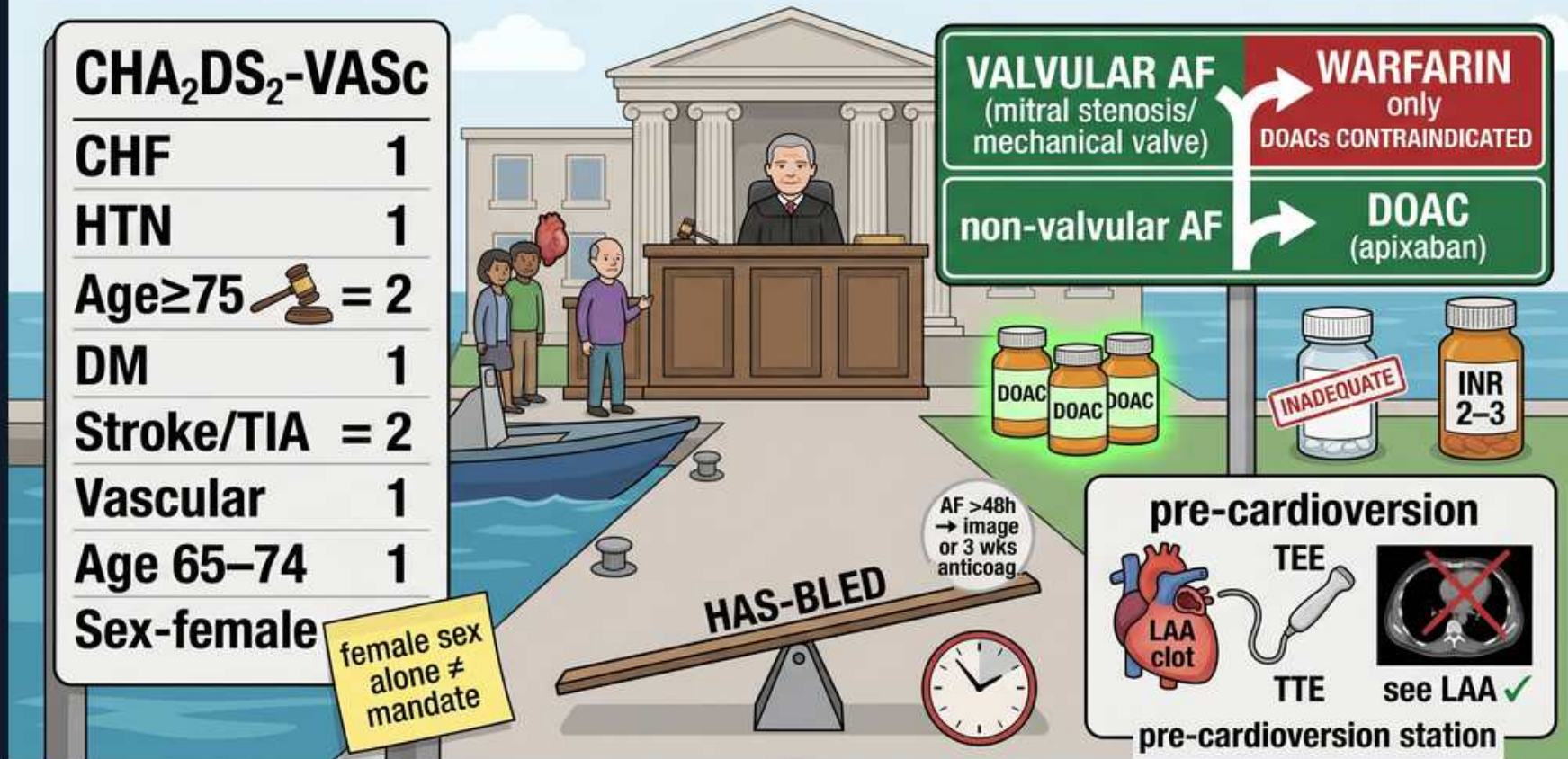


SCENE 1 — AF Anticoagulation: “The Stroke-Prevention Courthouse”



1. CHA₂DS₂-VASc scoreboard

WHAT YOU SEE

Left scoreboard listing CHF, HTN, Age, DM, Stroke, Vascular, Sex with point values.

WHAT IT ENCODES

CHA₂DS₂-VASc estimates annual ischemic-stroke risk in non-valvular AF and drives the anticoagulation decision. Anticoagulate when score ≥ 2 (men) or ≥ 3 (women); score 0 (men)/1 (women) can be left off. C/H/D/Vascular/Age65-74/female = 1 each; Age ≥ 75 and prior Stroke/TIA/thromboembolism = 2 each.

BOARD TRAP

WRONG: 'aspirin is adequate when the score is 1-2.' Antiplatelets are NOT a substitute — a true indication needs an oral anticoagulant (DOAC or warfarin).

2. Age ≥ 75 = 2 points

WHAT YOU SEE

Row 'Age ≥ 75 ' with a double gavel = 2.

WHAT IT ENCODES

Age ≥ 75 carries TWO points (the heaviest age weight) because stroke risk rises steeply with age. A single elderly patient with no other factor already reaches a treatment-level score.

BOARD TRAP

WRONG: 'every risk factor is worth 1 point.' Age ≥ 75 and prior stroke/TIA are each worth 2 — miscounting these changes the treatment decision.

3. Stroke/TIA = 2 points

WHAT YOU SEE

Row 'Stroke/TIA = 2'.

WHAT IT ENCODES

Prior stroke, TIA, or systemic embolism is the strongest predictor and scores 2 points — secondary prevention. Any such history alone mandates lifelong anticoagulation regardless of other factors.

BOARD TRAP

WRONG: 'a remote TIA doesn't count.' Prior cerebral ischemia is a 2-point, treatment-defining factor.

4. Female sex alone $\neq$ mandate

WHAT YOU SEE

Sticky note: 'female sex alone $\neq$ mandate'.

WHAT IT ENCODES

Female sex is a risk MODIFIER, not an independent trigger: a woman with sex as her only point (score 1) does not require anticoagulation. The threshold is ≥ 2 in men and ≥ 3 in women so that sex alone never tips the decision.

BOARD TRAP

WRONG: 'any woman with AF needs anticoagulation.' Female sex counts only when other risk factors are present.

5. Valvular AF → warfarin only

WHAT YOU SEE

Top road sign: VALVULAR AF (mitral stenosis/mechanical valve) → WARFARIN, DOACs CONTRAINDICATED.

WHAT IT ENCODES

'Valvular AF' = moderate-to-severe mitral stenosis or a mechanical heart valve. These REQUIRE a vitamin-K antagonist (warfarin) — DOACs are contraindicated (RE-ALIGN showed harm with dabigatran in mechanical valves). CHA₂DS₂-VASc is not used here; the valve itself mandates warfarin.

BOARD TRAP

WRONG: 'a DOAC is fine for AF with a mechanical valve.' DOACs are contraindicated in mechanical valves and significant mitral stenosis — use warfarin.

6. Non-valvular AF → DOAC

WHAT YOU SEE

Sign: non-valvular AF → DOAC (apixaban).

WHAT IT ENCODES

For non-valvular AF, DOACs (apixaban, rivaroxaban, dabigatran, edoxaban) are first-line over warfarin — comparable/better stroke prevention with less intracranial bleeding and no INR monitoring. Apixaban is often favored in CKD and bleeding-prone patients.

BOARD TRAP

WRONG: 'warfarin is preferred for all AF.' Warfarin is reserved for valvular AF; otherwise DOACs are preferred.

7. DOAC bottles (apixaban)

WHAT YOU SEE

Three green-glowing DOAC bottles.

WHAT IT ENCODES

DOACs need no routine level monitoring and have rapid on/off kinetics. Apixaban dose-reduces to 2.5 mg BID with ≥ 2 of: age ≥ 80 , weight ≤ 60 kg, creatinine ≥ 1.5 ; dabigatran/edoxaban need adequate renal function.

BOARD TRAP

WRONG: 'DOACs always need INR checks.' DOACs are not monitored by INR — that's warfarin.

8. Aspirin = INADEQUATE

WHAT YOU SEE

Aspirin bottle stamped INADEQUATE.

WHAT IT ENCODES

Antiplatelet therapy (aspirin $\pm$ clopidogrel) is inadequate for AF stroke prevention and carries bleeding risk without the benefit — it is no longer recommended in place of anticoagulation.

BOARD TRAP

WRONG: 'aspirin is a safer alternative to anticoagulation in the elderly.' It is both less effective and not meaningfully safer — use a DOAC.

9. Warfarin INR 2–3

WHAT YOU SEE

Warfarin bottle with INR 2–3 target.

WHAT IT ENCODES

When warfarin is required (valvular AF), the target INR is 2.0–3.0 (2.5–3.5 for some mechanical valves). It needs bridging consideration and regular monitoring.

BOARD TRAP

WRONG: 'INR target is 1.5–2 for AF.' Sub-therapeutic INR <2 does not protect against stroke.

10. HAS-BLED seesaw

WHAT YOU SEE

A balance labeled HAS-BLED weighing stroke vs bleeding.

WHAT IT ENCODES

HAS-BLED estimates bleeding risk; a high score flags modifiable factors (BP, alcohol, labile INR, concomitant antiplatelets) to correct — it does NOT by itself contraindicate anticoagulation.

BOARD TRAP

WRONG: 'a high HAS-BLED means withhold anticoagulation.' It prompts risk-factor correction and closer follow-up, not withholding.

11. AF >48 h → image or 3 wks

WHAT YOU SEE

Clock: AF >48h → image or 3 weeks anticoagulation.

WHAT IT ENCODES

Before cardioversion of AF lasting >48 h (or unknown duration), either give ≥3 weeks of therapeutic anticoagulation OR exclude left-atrial-appendage thrombus first; anticoagulate ≥4 weeks after cardioversion regardless.

BOARD TRAP

WRONG: 'cardiovert immediately if the patient feels well.' Unanticoagulated cardioversion of AF >48 h risks thromboembolic stroke.

12. Pre-cardioversion: TEE/CT vs TTE

WHAT YOU SEE

Heart with LAA clot; TEE probe and CT see LAA ; TTE crossed out.

WHAT IT ENCODES

To exclude a left-atrial-appendage thrombus before cardioversion, use TEE (or cardiac CT) — a transthoracic echo (TTE) cannot reliably visualize the appendage.

BOARD TRAP

WRONG: 'a normal TTE rules out atrial thrombus.' TTE cannot see the LAA — you need TEE or CT.
